

PATIENT MEDICAL CHART

CONFIDENTIAL - FICTIONAL RECORD FOR AUTHORIZED SOFTWARE TESTING

1. PATIENT DEMOGRAPHICS

Field	Value
Patient Name	Monica Elise Chen
DOB / Age	January 12, 1982 / 44
MRN	GBS-104982
Sex	Female
Address	214 Seaside Park Dr, Tampa, FL 33602
Preferred Language	English
PCP	Dr. Nina Patel, MD

2. INSURANCE & COVERAGE

Field	Value
Payer	Cigna Open Access Plus (Commercial)
Member ID	CIG-88420139
Group	OAP-FL-2218
Coverage effective	January 1, 2026
Prior authorization	Required - bariatric surgery
Requested facility	Gulf Coast Metabolic Surgery Center

3. ACTIVE PROBLEM LIST

ICD-10	Diagnosis	Status
E66.01	Severe obesity due to excess calories	Active
Z68.33	BMI 33.0-33.9, adult	Active
E11.65	Type 2 diabetes with hyperglycemia	Active
E78.2	Mixed hyperlipidemia	Active
K21.9	GERD	Active

4. CURRENT MEDICATIONS

Medication	Dose	Frequency	Indication
Metformin ER	1000 mg	BID	Type 2 diabetes
Semaglutide	1 mg	Weekly	Diabetes / weight management
Atorvastatin	20 mg	Nightly	Hyperlipidemia
Omeprazole	20 mg	Daily	GERD

5. ALLERGIES

Allergen	Reaction	Severity
Sulfonamide antibiotics	Diffuse rash	Moderate

6. BARIATRIC SURGERY CONSULT - APRIL 2, 2026

- Chief complaint: persistent obesity and diabetes despite structured medical treatment.
- Height 5 ft 5 in; weight 200 lb; measured BMI 33.3 kg/m2. Weight has remained 196-205 lb over the last 12 months.
- Type 2 diabetes has remained above goal despite metformin, semaglutide, dietitian-directed nutrition, and exercise. Most recent HbA1c 8.1%.
- Proposed procedure: laparoscopic sleeve gastrectomy, CPT 43775. Surgeon discussed expected benefits, alternatives, risks, and lifelong follow-up.
- Patient denies active substance use disorder, uncontrolled psychiatric illness, or inability to follow postoperative care.

Physical examination

Field	Value
BP	132/78
HR	82
Weight	200 lb
BMI	33.3 kg/m2
Abdomen	Soft, non-tender; no hernia
Functional status	Independent; walks 30 minutes 5 days/week

Assessment and plan

- Adult with class 1 obesity and clinically significant obesity-related comorbidity (type 2 diabetes).
- Continue multidisciplinary preoperative evaluation. Obtain nutrition evaluation, mental-health clearance, and documentation of failed medical weight management within 12 months.
- Proceed with authorization after all components are complete.

7. MEDICAL WEIGHT-MANAGEMENT RECORD

Date	Intervention	Weight / Outcome
May-Aug 2025	Dietitian-supervised calorie deficit; food logging	203 to 198 lb, then regained
Sep-Dec 2025	Exercise prescription and behavioral coaching	Weight 198-202 lb
Jan-Mar 2026	Semaglutide plus metformin; monthly physician visits	Nadir 196 lb; current 200 lb; HbA1c remains 8.1%

Treating physician conclusion

After 11 consecutive months of supervised nutrition, physical activity, behavioral intervention, and anti-obesity pharmacotherapy, clinically meaningful and durable weight loss was not achieved.

8. NUTRITION EVALUATION - APRIL 16, 2026

- Registered dietitian reviewed dietary history, protein requirements, meal pacing, hydration, vitamin supplementation, and postoperative diet stages.
- Patient demonstrated understanding using teach-back and has already eliminated sugar-sweetened beverages.
- No nutritional contraindication to sleeve gastrectomy. Cleared from nutrition standpoint.

9. BEHAVIORAL HEALTH EVALUATION - APRIL 22, 2026

- PHQ-9 score 3; GAD-7 score 2. No history of psychosis, mania, current major depression, eating disorder, or substance-use disorder.
- Patient understands permanence of surgery, need for follow-up, and behavioral requirements.
- Unequivocal behavioral-health clearance: patient is an appropriate candidate for bariatric surgery.

10. LABORATORY AND DIAGNOSTIC DATA

Test	Result	Interpretation
HbA1c	8.1%	Above goal
Fasting glucose	164 mg/dL	Elevated
ALT	42 U/L	Mild elevation
Creatinine	0.8 mg/dL	Normal
TSH	1.9 mIU/L	Normal
Pregnancy test	Negative	No pregnancy

11. PREOPERATIVE SURGEON FOLLOW-UP - MAY 1, 2026

- Multidisciplinary evaluation completed within the prior 12 months.
- Surgeon confirms proposed laparoscopic sleeve gastrectomy (CPT 43775).
- Failure of medical weight management is documented. Nutrition evaluation and unequivocal mental-health clearance are present.
- Request submitted for treatment of obesity, not for an unrelated primary condition.

12. PLANNED PROCEDURE

Field	Value
Procedure	Laparoscopic sleeve gastrectomy
CPT	43775
Diagnosis	E66.01 / Z68.33 with E11.65
Requested date	June 18, 2026
PA status	Pending
Surgeon	Dr. Lucas Bennett, MD, FACS

Clinical indication summary

- Adult age 44.
- BMI 33.3 kg/m2.
- Clinically significant obesity-related comorbidity: type 2 diabetes.
- Documented failure of medical management.
- Procedure description, nutrition evaluation, and mental-health clearance all completed within 12 months.